



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : نادية سيد محمد الشرييني
المحافظة : الجيزة
الحى / المدينة : امبابة
رقم البطاقة : 28002262103402 رقم التليفون : 01143281777
العنوان : امبابة - الجيزة
العمر (تاريخ الميلاد) : 26-02-1980 المهنة : بدون عمل
تاريخ تسجيل الإستمارة : 09-10-2026

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (.....)
المقرر بما فيه
التوقيع: (.....)

التشخيص الإكلينيكي : ورم خبيث بالرحم
نتائج الأبحاث : (مرفق)
التشخيص النهائي : ورم خبيث بالرحم
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [6665] قسم العلاج : الاورام
التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة:
الوظيفة:
الوظيفة:
التوقيع:
التوقيع:
التوقيع:
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / د. / د. / د. /

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

Name: نادية سيد محمد الشريف
National ID: 28002262103402



تقرير طبي

السيد

تحية طيبة وبعد,,

بشأن تقرير طبي للمريض

إيماء للطلب المقدم من

رقم طبي:

بقسم:

نتشرف بان نوضح فيما يلي تقريراً عن حالته

تاريخ وساعة الخروج :
PM 01:17 05-07-2026

تاريخ وساعة الدخول :
-- --:-- yyyy-mm-dd

وبعد الكشف والفحوصات والمتابعة وجد انه

A patient of uterine cancer for Further investigations and imaging

الوقت :

-- --:--

التاريخ :

05-07-2026

الطبيب المعالج :
د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

تأدية
سيد محمد الشربيني
٩٥٧٣ الحى ٨
المقطم - القاهرة

١٩٨٠ / ١٠ / ١٠
٢٨٠٠٢٢٦٢١٠٣٤٠٢
KJ8657232

✓



٢٠٢٤/٠١ ٢٨٠/٢٢٦٢١٠٣٤٠٢

حاصلة على بكالوريوس معهد

ادارة وسكرتارية

أنثى مقيمة

احمد نصر عبدالمالك رئيس

البطاقة مارية حتى ٢٠٣١/٠١/٣٠



١



Name: نادية سيد محمد		
MR. : 8474		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها The multidisciplinary team discussion in August 2024 reviewed the patient’s pathology, which demonstrated triple-negative breast cancer (TNBC) with a pT2N0 stage. The consensus was to consider adjuvant therapy with capecitabine for a planned eight-cycle course. In March 2026 the patient underwent placement of a port. A recent three-day ultrasound of the abdomen and pelvis identified a 10-mm endometrial polyp and atrophic ovaries; magnetic resonance imaging of the abdomen and pelvis and tumor marker studies were ordered for further evaluation. The current plan is to proceed with the proposed capecitabine regimen pending completion of imaging and laboratory assessment.		
تحريرا في ,		
	التاريخ :	20-09-2026
	الوقت :	
	نهي سعيد علي محمد	الطبيب المعالج :
يعتمد ,		



Patient Name	: الأستاذة / نادية سيد محمد الشربيني	Request No.	: 13005114
Patient No.	: 28002262103402	Request Date	: 22/07/2024
Path No.	: 2534/24	Verification Date	: 12/10/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 44 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer, Post NACT.

Nature of Specimen: Left modified radical mastectomy specimen.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Results:

- * IPT (Positive control showed appropriate reactivity):
- ER: No nuclear reaction in tumor cells (score 0/8).
- PR: No nuclear reaction in tumor cells (score 0/8).
- Her2: No membrane reaction tumor cells (score 0).
- Ki67: 40 % nuclear reaction of tumor cells.

Conclusion:

Left breast cancer Post NACT, MRM specimen:

Residual invasive duct carcinoma, Grade III with focal DCIS .

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).
- Ki67: Positive in 40% of tumor cells.

For further IPT (GATA3)

* Scoring system: "ER, PR"

Verified by :

Dr. Sagda El-Kady

Printing Date : 14/12/2024 09:47:08

Page 1 of 2

Printed By : Mrs.Mayada Hegazy





Patient Name	الأستاذة / نادية سيد محمد الشربيني	Request No.	13005114
Patient No.	28002262103402	Request Date	22/07/2024
Path No.	2534/24	Verification Date	12/10/2024
Sex	Female	Financial Category	اورام دار السلام - نفقة الدولة
Age	44 Y	Department	Out Patient
Referring Doc.		Registered by	.marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ASCO-CAP ER/PR Test Guideline Recommendations, 2010 Allred score: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Signature

Dr. Marwa Mahmoud

Dr. Sagda Elkady

Verified by :

Dr. Sagda El-Kady

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Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذ / نادية سيد محمد الشربيني	Request No.	: 13005114
Patient No.	: 28002262103402	Request Date	: 22/07/2024
Path No.	: 2534/24	Verification Date	: 10/08/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 44 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left modified radical mastectomy specimen.

Gross Examination:

- * Procedure: Left MRM measured 22x17x7 cm with covering skin ellipse measured 17x8 cm.
- * Tumor extent: A greyish white firm tumor bed site.
- * Size: 4.5 x4 x3 cm.
- * Skin involvement: Nipple & areola were unremarkable grossly.
- * Distance from margins: 0.7 cm away from deep margin.
- * Lymph nodes number and size: 29 L.Ns up to 3.3x3cm.

Microscopic Examination:

- * Histological type: Residual Invasive duct carcinoma.
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
- Maximal tumor diameter 4.5x4x3 cm.
- Actual tumor cellularity: 5%, tumor necrosis index: 95%.
- * Ductal carcinoma in situ: cribriform and papillary.
- * Skin involvement: Free Nipple and areola.
- * Lymph node involvement: Positive in 5/29.
- * Resection Margins: Free deep margins.
- * Histologic response to therapy: Partial pathological response.

Diagnosis:

- Left breast cancer post NACT, MRM specimen:
Residual Invasive duct carcinoma Grade III with focal DCIS.
- Maximal tumor diameter: 4.5x4 cm
 - Actual tumor cellularity: 5%, tumor necrosis index: 95%.
 - Free deep margin.

Verified by :



Patient Name : الأستاذة / نادية سيد محمد الشربيني

Request No. : 13005114

Patient No. : 28002262103402

Request Date : 22/07/2024

Path No. : 2534/24

Verification Date : 10/08/2024

Sex : Female

Financial Category : نفقة الدولة - اورام دار السلام

Age : 44 Y

Department : Out Patient

Referring Doc :

Registered by : marwa mohamed youss

Pathology Report

- Free nipple and areola.
- Positive lymph nodes in (5/29)
- Partial pathological response.
- Stage: ypT2, ypN2.
- RCB: 3.589, RCB -III class .

Signature
Dr. Walaa Ghonim.
Dr. Sagda Elkady.

Verified by :

Printing Date : 10/08/2024 11:09:15

Page 2 of 2

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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt



Patient Name	: الأستاذ / نادية سيد محمد الشربيني	Request No.	: 13002673
Patient No.	: 28002262103402	Request Date	: 28/01/2024
Path No.	: SR-128/24	Verification Date	: 03/02/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 43 Y	Department	: Out Patient
Referring Doc :		Registered by	: Mrs.Aya Mohamed

Pathology Report

Clinical Data: Left breast cancer.
Nature of Specimen: Slide Revision

Gross Examination:

- Two paraffin blocks labeled by (200125/23) and three slides were received.

Microscopic Examination:

- * Histological type: Invasive duct carcinoma
- * Grade (differentiation+ nuclear pleomorphism+ mitosis): (3+3+2) Grade III.
- * Ductal carcinoma in situ: Focal
- * Perineural invasion: No
- * Angioinvasion: Positive
- * IPT:
- ER: No nuclear reaction in tumor cells (Allred score 0/8).
- PR: No nuclear reaction in tumor cells (Allred score 0/8).
- Her2: No membrane reaction in tumor cells (score 0).

Diagnosis:

Left breast mass, Slide Revision:
Invasive duct carcinoma, Grade III with focal DCIS.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).
- Her2: Negative (score 0).

Signature
Prof. Dr. Amal Hareedy
Dr. Heba Hasan

Verified by :

Dr.Marwa mahmoud

Printing Date : 03/02/2024 11:14:12

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Printing By : Ali mostafa



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Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

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Name : Nadia Sayed Mohamed Al Sherbiny
Branch : Dokki

Date : 2024-01-16
Medical No. : 110615077

DYNAMIC MR EXMAINATION OF BOTH BREASTS

Technique

- Plate 1: Axial T1 WI/SE
- Plate 2: Axial STIR
- Plate 3: Axial diffusion
- Plate 4: Axial SPIR and gadolinium
- Plate 5: sagittal SPIR and gadolinium
- Plate 5, 6 & 7: dynamic post gadolinium post subtraction series
- Plate 8: Time /intensity curve

Technical presentation:

Left breast mastectomy with breast reconstruction.
Right breast mass.

Findings:

- Prominent fibro glandular left breast tissue showing marked homogenous background parenchymal enhancement.

Left breast

- LIQ at about 8-9 o'clock an ill-defined mass is seen eliciting hypointense T1 & intermediate signal in T2 WIs as well as restricted diffusion on DWIs. After contrast injection the mass shows heterogenous mass enhancement measuring 15x18mm.
- Deeply seated LIQ at about 7 o'clock pre-pectoral two closely related well circumscribed heterogeneously enhancing masses are seen measuring 5x6mm and 9x10mm? Multifocal masses? Pathological intra mammary LNs.
- Associated mild interstitial breast oedema and areolar skin thickening are also noted.
- Left pathologically enlarged level I axillary LNs, the largest measures 16x20mm.
- Enlarged internal mammary LN is seen measuring 7x8mm.

Right breast

- No enhancing masses seen.
- No pathologically enlarged axillary LNs.

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Name : Nadia Sayed Mohamed Al Sherbiny

Date : 2024-01-16

Branch : Dokki

Medical No. : 110615077

Opinion:

- Left breast locally advanced carcinoma likely multifocal with ipsilateral pathological axillary LNs as well as enlarged internal mammary LN (BIRADS 5).
- No right breast enhancing lesions (BIRADS 2).

Dr .Rasha Wessam ,MD

N I L E
D I A G N O S T I C
C E N T E R

Dr. Ahmed El-Habashi, MD, PhD, FIAC
Professor of Pathology;
NCI, Cairo University
International Board of Cytopathology
Fellow of International Academy of Cytology
Fellow Tulane Univ. Hospital, USA
Fellow Groningen Univ. Hospital, Holland

أ.د. أحمد حسين الحبشي
أستاذ الباثولوجي - المعهد القومي للأورام - جامعة القاهرة
بورد السيثوباثولوجي الدولي
زميل الأكاديمية الدولية للسيثولوجي
زميل جامعة تولين- نيو اورليانز- امريكا
زميل جامعة خرونيجن- هولندا

Patient Name: نادية سيد محمد

Gender: Female

Age: 47 Y-O

Clinical History: Endometrial hyperplasia. H/O of bilateral mastectomy and chemotherapy

Nature of Specimen: Endometrial biopsy

Referring Prof. Doctor: Emad Salah

Date of Specimen Receiving: 20/9/2026

Date of Reporting: 23/9/2026

Surgical Pathology/ Cytopathology Report

Gross Description:

Specimen received in formalin and consisted of hemorrhagic fragments of soft tissue 2.5x2 cm in aggregates. Sections in one cassette.

Microscopic Description:

Sections showed malignant tumor formed of solid patterns, irregular closely packed tubular, cribriform patterns of epithelial cells displaying moderate to marked anaplasia manifested by nuclear stratification, loss of polarity, high N/C ratio, prominent nucleoli and high mitotic activity. Focal necrosis, apoptosis and neutrophilic cell infiltrate.

Diagnosis

Endometrial Biopsy:

Adenocarcinoma, grade II-III (primary versus metastatic).
For immunophenotyping.



Ahmed El-Habashi, MD, FIAC

10 شارع صدري أبو علم بحوار وزارة الاوقاف - باب اللوق - القاهرة

تليفون : 23929937 موبيل : 0166736608

E-Mail: elhabashiahmed@yahoo.com

Dr. Ahmed El-Habashi, MD, PhD, FIAC
Professor of Pathology;
NCI, Cairo University
International Board of Cytopathology
Fellow of International Academy of Cytology
Fellow Tulane Univ. Hospital, USA
Fellow Groningen Univ. Hospital, Holland

أ.د. أحمد حسين الحبشي
استاذ الباثولوجي - المعهد القومي للأورام - جامعة القاهرة
بورد السيثوباثولوجي الدولي
زميل الأكاديمية الدولية للسيثوباثولوجي
زميل جامعة تولين- نيو اورليانز- امريكا
زميل جامعة خروينجن- هولندا

Patient Name: نادية سيد محمد

Gender: Female

Age: 47 Y-O

Clinical History: Endometrial adenocarcinoma

Nature of Specimen: Endometrial biopsy, Immunocytochemistry

Referring Prof. Doctor: Emad Salah

Date of Specimen Receiving: 29/9/2026

Date of Reporting: 4/10/2026

Immunocytochemistry Report

Gross Description:

Smears were immunocassayed for GATA-3, PAX-8, ER, PR, HER2 and Ki67 Using ABC technique with antigen retrieval and DAB chromogen.

Microscopic Description:

Tumor cells showed positive immunoreactivity for GATA-3 and PR and negative for PAX-8 and ER. HER2 showed positive membranous staining in >10% of cells. Ki-67 stained 70 % of tumor cells.

Diagnosis:

Endometrial Biopsy. Immunohistochemistry

Immunophenotypic pattern compatible with metastatic high grade adenocarcinoma, of primary breast origin (GATA-3 positive andPAX-8 negative).

Estrogen Receptor: Negative (score 0/8).

Progesterone Receptor: Positive (3+, score 7/8).

Her-2/neu Receptor: Positive (Score 3+)

Ki67 labeling index: 70%

Comment:

Immunostainings results should be considered in the context of total amount of relevant data including clinical, radiological and tumor marker serum level findings.

P.S: 6 slides enclosed.


Ahmed El-Habashi, MD, FIAC

Name	Mrs. Nadia Sayed Mohamed Al-Sherbiny		
Ref. By Prof. Dr.	Amr Kamal	Date	3/10/2026
ID	7726012952	Acc. No.	291374323

7726012952

18F-FDG PET/CT Scan

Indication of the study: Status evaluation study for a case with previous history of breast neoplasm, developed endometrial adenocarcinoma (pathology 9.2026). No available previous studies.

Head & Neck:

- Normal physiological biodistribution over scanned portion of the brain.
- (N.B; not all brain metastatic lesions could be detected on 18-FDG PET/CT due to physiologic uptake in brain. MRI brain is suggested in cases of clinical suspicion).
- Normal naso-/oro-pharynx, larynx, thyroid (Non-homogenous thyroid gland, for further thyroid work-up) and salivary glands.

- *Metabolically active bilateral supra-clavicular lymph nodes, largest about 1.2 cm of SUVmax 4.5.*

Torso:

Thorax:

- *Low activity management bed reticulations.*
- *No focal breast active masses.*
- *No enlarged FDG hypermetabolic mammary related lymph nodes.*
- *For mammographic correlation.*
- *Metabolically active bilateral hilar and mediastinal (all groups) lymph nodes, largest about 2.5 cm of SUVmax 7.6.*
- *Left pulmonary post irradiation changes.*
- *Low activity minimal bilateral pleural effusions.*
- *Few bilateral sub-pleural non-metabolically active nodules, largest $\pm$ 4-6 mm, mostly non-specific, however, for follow up.*

- Bilateral tracer starved atelectasis and sub-pleural reticulations.

- No FDG hypermetabolic pulmonary nodules or masses seen.

Abdomen & Pelvis:

- Enlarged liver with non-uniform activity yet no FDG hypermetabolic focal lesions,

mostly blood pooling. For dynamic MRI study of the liver assessment if warranted.

Total body PET scan was done 40 minutes after IV administration of 300 MBq of 18F-FDG.

CT scanning was performed simultaneously without IV contrast material injection.

Semi-quantitative analysis would be done for selected ROIs & the normal threshold would be the normal blood pool liver.

S. checked by: Dr. L.A.

Report by: Dr. Ali

مركز الطاهرة للشعة والتحاليل الطبية
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ACCREDITED
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G-CERT
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Client No: GCG-0182-0C

Name	Mrs. Nadia Sayed Mohamed Al-Sherbiny		
Ref. By Prof. Dr.	Amr Kamal	Date	3/10/2026
ID	7726012952	Acc. No.	291374323

- Average size of the spleen with no focal active lesions.
- Normal appearance of pancreas (no sizable active masses) and adrenals.
- *Metabolically active para-aortic, circumcaval, bilateral iliac, internal and external iliac lymph nodes, largest about 2.5 cm of SUVmax. 11.7.*
- *Bulky uterus with increased activity eliciting SUVmax. 15.*
- Congested mesentery with strandy reticulations.
- Normal physiological biodistribution over the bowel (non-uniform bowel uptake with no sizable masses) & urinary tracts.

Surveyed skeleton:

- Faint marrow uptake of SUVmax. 2.8 yet no tracer active lytic or sclerotic osseous lesions seen, likely hematopoietic, for CBC correlation and follow up.
- Scattered sclerotic non-metabolically active foci of the spine and pelvic bones (mostly degenerative changes).

Opinion:

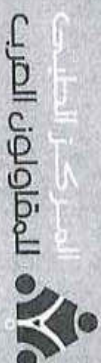
- Active endometrial thickening (would fit with the provided data), for contrast enhanced MRI pelvis evaluation.
- Active nodal lesions (metastatic deposits).
- For mammographic correlation.

Best Regards;

Dr. Islam Aboulwafa, MD



ARAB CONTRACTORS MEDICAL CENTER



أطابق جريدة للرجعية الطبية

Patient ID: 1137707	Service Name: Post contrast MRI pelvis
Patient Name: Nadia Sayed Mohamed ALSHRBINI	Service Date: 10/09/2026
Patient Age: 46 Y	

Clinical data:

- 46 years old female known treated bilateral breast cancer.
- Finished chemo and radiotherapy.
- Period isn't regular since chemo therapy.
- Now presented with vaginal spotting.



POST CONTRAST M.R.I STUDY OF THE PELVIS

MR Technique:

- Axial T1 and T2 weighted images.
- Sagittal T2 weighted images.
- Coronal T1 and T2 weighted images.
- DWIs and ADC.
- Post contrast imaging.
- Complementary TVUS was done.



MR Findings:

- The hall mark of the examination is distended uterine cavity by a fairly defined mass lesion which masks proper assessment of the endometrium, the endometrium and junctional zone couldn't be traced intact all through. The lesion elicits low signal in T1, inhomogenously indeterminate signal in T2 WIs, and appears faintly restricted in DWIs and ADC. It shows no definite contrast uptake compared to the surrounding myometrium. It roughly measures about 2.5 x 3.1 x 2.8 cm in its maximal CC, AP, TS dimensions respectively.....attached 1st and 2nd image.....Includes several differentials; Endometrial polyp.....sub mucous fibroid.....?? neoplastic, better for histo pathological correlation.
- Small sized both ovaries with no sizable adnexal masses or cysts.
- Bilateral enlarged iliac lymph nodes more in number and larger in size on the left side, most sizable is seen lateral to left internal iliac vessels and measures about 2.5 x 1.5 cm in its maximal axial dimensions.....attached 3rd image.
- No sizable pelvic collection.
- Normal MR appearance of the pelvic bones.

PROF. TAMER KAMAL
Dr. Ola Abounour, MD

